RESPONSE TO REQUEST FOR INFORMATION

Accelerating the Adoption and Use of Artificial Intelligence
as Part of Clinical Care

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Submitted By:	COGA Labs
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Executive Summary

COGA Labs develops **AI-powered stress detection technology** that uses smartphone sensors and proprietary machine learning models to identify acute and chronic stress states in real time. Our platform analyzes behavioral signals and physiological indicators to provide actionable insights for individuals and the professionals supporting them.

Our founding team combines hands-on AI product development experience (including building and shipping production AI systems for enterprise clients) with management consulting (BCG) and financial services (Deutsche Bank) backgrounds. This positions us to comment on both the technical and commercial barriers to AI adoption in clinical care.

We welcome the opportunity to share practitioner-level perspectives with HHS as it shapes policy to accelerate responsible AI adoption in health care.

Response to Question 1: Barriers to Private Sector AI Innovation in Health Care

“What are the biggest barriers to private sector innovation in AI for health care and its adoption and use in clinical care?”

1. Regulatory Classification Ambiguity

The FDA’s evolving framework for Software as a Medical Device (SaMD) creates significant uncertainty for AI companies working at the intersection of wellness and clinical care. Products that analyze behavioral or physiological data for stress, mental health, or cognitive performance sit in a gray zone between “wellness” and “diagnostic” classification. This ambiguity delays market entry and diverts capital from product development to regulatory navigation.

Recommendation: HHS should consider establishing an “AI Health Sandbox”—modeled on regulatory sandbox programs in the UK (MHRA AI Airlock) and Singapore—that grants provisional authorization for AI health tools to operate in supervised clinical environments while generating real-world evidence for full clearance.

2. No Clear Reimbursement Pathway

There is currently no straightforward CPT code pathway for AI-augmented mental health screening or continuous behavioral health monitoring. Clinicians who adopt these tools absorb the cost, creating negative ROI regardless of clinical value. Until reimbursement catches up, adoption will remain limited to self-pay wellness markets rather than mainstream clinical care.

Recommendation: CMS should establish provisional reimbursement codes for FDA-authorized AI clinical tools, tied to outcomes-based payment models with built-in sunset provisions to allow data collection without permanent cost commitment.

3. Prohibitive Integration Costs

For small AI companies, the cost of integrating with legacy EHR systems remains a major barrier. FHIR adoption is growing but incomplete, and each health system integration requires custom engineering work that favors large incumbents over innovative startups. This concentrates AI innovation among companies with existing health system relationships rather than those building the best technology.

Recommendation: HHS should incentivize EHR vendors to establish standardized API marketplaces where FDA-authorized AI tools can be deployed within existing clinical workflows, reducing the per-integration cost that currently locks out smaller innovators.

4. Data Access for Model Training

AI models require diverse, representative data to achieve clinical-grade performance. However, the combination of HIPAA's minimum necessary standard, fragmented data systems, and proprietary formats makes it disproportionately expensive for small companies to access sufficient clinical training data. Expanding the ONC's Trusted Exchange Framework (TEFCA) to include a specific pathway for AI development—with standardized de-identification and pre-negotiated data use agreements—would significantly lower this barrier.

Response to Question 8: Interoperability Opportunities

"Where would enhanced interoperability widen market opportunities, fuel research, and accelerate the development of AI for clinical care?"

Behavioral and mental health data remains the most fragmented domain in clinical informatics. Assessment scores, therapy notes, medication adherence records, and AI-derived behavioral indicators exist across disconnected systems with no unified patient view. A standardized FHIR-based mental health data profile—covering both structured clinical assessments and AI-derived behavioral markers—would enable longitudinal monitoring that is currently impossible at scale.

Wearable and sensor data standards are equally critical. Consumer devices generate clinically relevant physiological data (sleep patterns, heart rate variability, activity levels) that rarely reaches the clinical record. Standardized ingestion pipelines—building on IEEE 11073 and Open mHealth schemas—would create new training datasets for AI while enabling passive, continuous health monitoring. This is especially relevant for mental health and stress-related conditions, where longitudinal data provides better predictive value than point-in-time assessments.

Federated learning infrastructure deserves investment. Enhanced interoperability need not mean centralized data. A permanent, HIPAA-compliant federated learning network—accessible to small companies, not just academic medical centers—would accelerate AI model development while preserving privacy. The NIH's N3C project demonstrated the value of multi-institutional collaboration; a standing equivalent for health AI would be transformative.

Response to Question 9: Patient and Caregiver Perspectives

"What challenges do patients and caregivers wish to see addressed by AI in clinical care? What concerns do they have?"

From user research conducted during our product development across wellness and corporate health populations, we consistently observe two themes:

Demand for objective measurement: Users express frustration with subjective self-report tools (PHQ-9, GAD-7) that depend on memory and self-awareness. They actively seek AI tools that provide objective, data-driven indicators of their mental and physical state. In our experience, users who receive objective health data show significantly higher engagement with health and wellness resources. This suggests strong latent demand for AI-augmented health screening—provided the tools are transparent about methodology.

Data privacy as the dominant concern: Across our user base, data access and control consistently ranks as the top concern—ahead of accuracy, cost, or usability. Users are willing to share AI-derived health insights with their care providers but want explicit control over who sees their data, how long it is retained, and whether it can be used for secondary purposes. Clear HHS guidance on AI health data governance—distinct from general HIPAA provisions—would accelerate patient trust and adoption.

About the Respondent

COGA Labs is a health technology company developing AI-powered behavioral analysis tools for stress detection and mental wellness monitoring. Our team brings experience across AI product development, enterprise SaaS deployment, management consulting, and financial services. We have validated our core technology with hundreds of users and are actively pursuing clinical application pathways.

Relevant NAICS codes: 541512 (Computer Systems Design Services), 541519 (Other Computer Related Services). Small business.

COGA Labs welcomes follow-up consultation with HHS program offices and is available for briefings or technology demonstrations. Inquiries may be directed to the company through its website or public contact channels.

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